Cardiac Arrest

- Seek clinical advice if VF or VT persists.

If torsades de pointes is present or has occurred

- Withhold amiodarone.
- Administer magnesium IV:
  - 10 mmol (2.5 g) over 1-2 minutes for an adult.
  - See the paediatric drug dose tables for a child.
  - Repeat once if torsades de pointes recurs or persists.

If the patient is in PEA

- Correct immediately reversible causes.
- Administer 0.9% sodium chloride IV:
  - 1-2 litres IV for an adult.
  - 20-40 ml/kg IV for a child.

If patient movement is interfering with resuscitation

- Administer 1 mg/kg of ketamine IV, up to a maximum of 150 mg.
- This may be repeated once if required.
- If significant movement continues, consider administering 2 mg/kg of rocuronium IV up to a maximum of 200 mg, provided an ETT has been placed, and the position confirmed using $ETCO_2$.

Backup

- Backup from an ICP/CCP should be requested.

Cardiac arrest checklist

- ✓ Team leader identified.
- ✓ Roles allocated and confirmed.
- ✓ CPR feedback device and metronome on if available.
- ✓ Defibrillator screen visible to the team leader.
- ✓ Airway adjunct or ETT in place and ventilation adequate.

- ✓ $ETCO_2$ attached and waveform visible.
- ✓ Oxygen flow and supply adequate.
- ✓ IV/IO access secure, two sites if possible.
- ✓ Consider underlying and reversible causes.
- ✓ Appropriate medicines administered.
- ✓ Initiate documentation.
- ✓ Communication with family/bystanders.

Additional information

Deciding to start resuscitation

- Resuscitation should start unless there is a clear reason not to. Clear reasons for not starting resuscitation include:
  - » Signs of rigor mortis or post-mortem lividity.
  - » A clear advance directive not to receive resuscitation for cardiac arrest.
  - » Scenarios where resuscitation is futile or clearly not in the best interest of the patient. Examples include unwitnessed cardiac arrest with asystole as the initial rhythm, patients who are dying from cancer and patients with severe life-limiting disease.
- Ambulance personnel must consider all available information, including the patient's known wishes (for example, their advance directive) and act in what they believe is the best interest of the patient.
- If there is doubt regarding the appropriateness of a resuscitation attempt, resuscitation should begin while further information is gained.
- Nobody has the right to demand resuscitation on behalf of a patient, including those with an Enduring Power of Attorney and family/whānau, but the known wishes of the patient must be considered.
- Family/whānau do not have the right to decline resuscitation on behalf of a patient unless they have an Enduring Power of Attorney. See the **'treatment and/or transport without informed consent'** guideline for more information.
- There must be clear documentation regarding decisions made.